



Stroke Medical Director

Comprehensive Stroke Center (CSC)

Aligned with AHA/American Stroke Association Guidelines and Quality Initiatives

Position Summary

Provides physician leadership for the hospital's Comprehensive Stroke Center (CSC) program, holding ultimate clinical and administrative authority for stroke care quality, protocol development, and regulatory compliance. Works in tandem with the Stroke Coordinator, who typically manages day-to-day nursing/program administration, but carries the physician-level authority needed to drive clinical practice change, credential providers, and represent the program to medical staff and certifying bodies. A full glossary of abbreviations used throughout this description appears at the end of the document.

Reporting Relationships

- **Reports to:** Chief Medical Officer, VP of Medical Affairs, or Department Chair (varies by facility), often with a matrixed relationship to hospital administration for program resourcing
- **Works closely with:** Stroke Coordinator, ED Medical Director, Neurointerventional Radiology, Neurosurgery, Neurocritical Care, Rehabilitation Services, Quality/PI department, Medical Staff Office, and EMS medical direction
- **Clinical authority over:** Stroke protocol approval, physician credentialing/privileging for stroke call coverage, and final say on complex or disputed treatment decisions

Key Responsibilities — Detailed Breakdown

1. Clinical Leadership & Protocol Authority

- Serve as the physician champion and final clinical authority for all stroke-related protocols, order sets, and treatment algorithms.
- Lead or co-lead the multidisciplinary Stroke Committee alongside the Stroke Coordinator, providing physician-level sign-off on protocol changes.
- Make final determinations on complex or disputed treatment decisions — thrombolytic eligibility in borderline cases, EVT candidacy for large-core or basilar occlusion patients, and management of uncertain-onset or wake-up strokes.
- Provide direct clinical care or supervision for stroke patients; scope varies by facility — many Directors maintain active clinical practice in vascular neurology, neurology, neurosurgery, or emergency medicine alongside the Director role.
- Serve as a real-time consultative resource for ED physicians, hospitalists, and advanced practice providers managing acute stroke cases.
- Review and approve updates to thrombolytic agent protocols, EVT eligibility criteria, and extended treatment-window pathways as evidence evolves.

2. Program Oversight & Strategy

- Set the overall clinical vision and strategic direction for the stroke program, in partnership with the Stroke Coordinator and hospital administration.
- Oversee credentialing and privileging of physicians who participate in stroke call coverage (neurology, neurosurgery, neurointerventional radiology, neurocritical care).
- Ensure 24/7 physician coverage requirements are met for CSC-level certification, including backup/escalation coverage plans.
- Approve major protocol changes and evidence-based practice updates before implementation.
- Partner with hospital administration on program resourcing, staffing, and equipment/technology needs (e.g., imaging capability, neuro-IR suite, neuro ICU bed capacity).
- Provide physician-level input on the hospital's role in the regional hub-and-spoke stroke system, including transfer agreements with referring facilities.

3. Quality & Data

- Co-review stroke program performance data (GWTG-Stroke registry, Target: Stroke metrics, door-to-needle/door-to-device times) with the Stroke Coordinator.
- Lead physician-level case reviews for outlier cases, complications, or mortality/morbidity review.
- Champion performance improvement (PI) initiatives requiring physician buy-in or practice change, lending clinical authority to drive adoption.
- Present clinical outcomes and program performance to the medical staff, hospital board, and stroke committee.
- Review functional outcome data (modified Rankin Scale at discharge/90 days) and procedural complication rates to identify clinical practice improvement opportunities.

4. Regulatory & Survey Readiness

- Serve as the physician face of the program during Joint Commission/DNV certification surveys, answering clinical practice questions directly.
- Ensure medical staff bylaws, credentialing files, and physician competency documentation meet certifying body standards.
- Maintain personal continuing medical education (CME) in stroke care to meet Medical Director eligibility requirements.
- Support the Stroke Coordinator in preparing the certification survey binder — specifically physician-side documentation (credentialing, privileging, coverage schedules).
- Ensure compliance with state-specific CSC/Level I stroke center designation requirements from the physician leadership side.

5. Education & Mentorship

- Provide physician-to-physician education on stroke protocols and evidence updates, including grand rounds and case-based teaching.
- Mentor and support ED physicians, hospitalists, and advanced practice providers in stroke recognition and initial management.
- Represent the program in community and professional education — grand rounds, EMS medical direction, regional/national conferences.
- Provide medical oversight and education input for EMS prehospital stroke protocols and large vessel occlusion (LVO) recognition training.
- Mentor junior faculty or residents/fellows involved in stroke care, where the facility has an academic affiliation.

Work Environment & Physical Demands

- Hospital-based, with time split between clinical duties (if maintaining active practice), committee leadership, administrative meetings, and case review.
- May require on-call availability for complex case consultation or escalation beyond the standard stroke call schedule.
- Regular interaction with hospital executive leadership, medical staff office, and certifying body surveyors during accreditation cycles.
- Frequent movement between clinical areas (ED, neuro ICU, neuro-IR suite) and administrative/committee settings.
- Time commitment often structured as a defined percentage of protected administrative time (e.g., 0.2–0.5 FTE) in addition to clinical duties.

Qualifications

- **Education/Training:** MD or DO required; residency in Neurology, Neurosurgery, Emergency Medicine, or Internal Medicine; fellowship training in Vascular Neurology or Neurocritical Care strongly preferred, especially at the CSC level.
- **Board Certification:** Board certification in the relevant specialty (e.g., Vascular Neurology, Neurology) required or strongly preferred.
- **Licensure:** Active, unrestricted medical license in the state of practice.
- **Experience:** Active clinical practice in stroke/cerebrovascular care; prior stroke program leadership or committee experience preferred.
- **Clinical knowledge:** Deep working knowledge of AIS and hemorrhagic stroke management, thrombolytic and EVT eligibility criteria, and neurocritical care principles at the CSC level of complexity.
- **Leadership & communication skills:** Demonstrated ability to drive clinical practice change across departments, lead physician peers, and communicate program performance to executive leadership and the board.
- **Data literacy:** Comfort interpreting GWTG-Stroke registry data, time-target performance metrics, and functional outcome measures to guide clinical decision-making.

Glossary of Abbreviations

- **AHA** — American Heart Association
- **ASA** — American Stroke Association
- **CSC** — Comprehensive Stroke Center
- **AIS** — Acute Ischemic Stroke
- **EVT** — Endovascular Therapy (mechanical thrombectomy)
- **GWTG-Stroke** — Get With The Guidelines–Stroke (AHA's national stroke data registry and quality program)
- **PI** — Performance Improvement
- **mRS** — modified Rankin Scale (a scale measuring degree of disability/dependence after stroke)
- **LVO** — Large Vessel Occlusion
- **ED** — Emergency Department
- **ICU** — Intensive Care Unit
- **EMS** — Emergency Medical Services
- **CME** — Continuing Medical Education
- **FTE** — Full-Time Equivalent (a measure of protected/allocated work time)
- **MD / DO** — Doctor of Medicine / Doctor of Osteopathic Medicine